

Chapter 06 Raw Textbook Content

Textbook of Minimally Invasive Spine Surgery (Springer Nature 2026) | Full Raw Text Extraction

Textbook of Minimally Invasive Spine Surgery, Concepts and Surgical Techniques

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6. Interlaminar Endoscopic Lumbar Discectomy

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6.1 Introduction

Lumbar disc herniation (LDH) is one of the most common degenerative spinal disorders. It may cause intractable low back pain, radiating leg pain, and varying degrees of neurological dysfunction and significantly affect the quality of life of patients. Conservative management, including rest, physiotherapy, medications, and epidural steroid injections, is effective in most patients. However, approximately 10% of patients do not achieve sufficient symptom relief and eventually require surgical intervention [1]. Open discectomy has been the standard and effective procedure for nerve decompression. However, it is associated with collateral damage to surrounding tissues, prolonged hospitalization, postoperative scar adhesions, and potential compromise of spinal stability, all of which have drawn increasing attention [2, 3].

The advancements in minimally invasive concepts and surgical instruments have led to the development of full-endoscopic spine surgery over the past three decades. This has enabled precise visualization and targeted surgical intervention with minimal tissue disruption [4, 5]. Transforaminal endoscopic lumbar discectomy (TELDD), which uses Kambin's triangle to access the disc and spinal canal, has become a widely adopted and effective treatment for LDH. It is minimally invasive, facilitates faster recovery and shorter hospital stays, and yields favorable clinical outcomes [4, 6, 7]. However, anatomical challenges such as a high iliac crest, hypertrophic facet